

AI BACKLOG KILLER

Agent-Patient Mapping Recovery Plan | Executive Brief for COO

Decision requested

Approve a 30-day recovery sprint to establish one controlled Agent-Patient mapping register, clear the highest-value backlog first, and prevent new unassigned referrals from accumulating.

1. Executive Summary

The visible backlog is approximately 1,800 agent records accumulated over approximately eight months. The underlying problem is broader: fragmented data, no common mapping rule, no visible queue or SLA, and single-person dependency. Manual one-by-one mapping will not scale and will recreate the backlog.

Current gap	Business impact	Recommended response
Agent identity and patient attribution are not consistently linked.	Unreliable agent performance, patient attribution, referral ROI, and campaign targeting.	Use a master register, priority queue, AI-assisted preparation, human approval, and a live control dashboard.
Measure	30-day target	
Agent records in scope	100% inventoried and classified	
Priority agents	136 reviewed; status visible	
Actionable backlog	Reduce by at least 50% after row-level denominator is confirmed	
New unassigned records	Zero older than 2 working days	
Quality	At least 95% accuracy in an audited sample before production use	

2. COO Approval Required

Approve: one accountable owner; one data steward/analyst and IT/system owner; the 136 priority agents as first queue; no campaign, benefit, or financial attribution from unverified mapping; a common evidence rule/status/SLA; and PDPA/DPO review before identifiable data is used in AI or campaign workflows.

3. Backlog Diagnosis

Dimension	Diagnosis	Design response
People	Previous single-person dependency; incomplete handover.	Named accountable owner, data steward, IT owner, sponsor.
Process	No standard definition of valid mapping, SLA, or closure evidence.	SOP, status codes, evidence hierarchy, daily queue review.
Data	Duplicates, spelling variants, incomplete codes, missing HN linkage.	Canonical Agent ID, standardisation, confidence score, exception queue.
Technology	Fragmented sources; bulk import capability not confirmed.	Controlled register, batch-load/API assessment, version history.
Decision/approval	Disputed attribution and privacy questions can stop the whole queue.	Separate preparation from approval; fixed decision gates and escalation SLA.

4. Priority Matrix

Scores are proposed and should be recalibrated after a 100-row sample. Effort is scored high when difficult.

Priority work package	I	U	R	E	AI	Action
136 priority agents	5	5	5	3	5	First queue; AI prepares, human approves
Recent/high-value/high-volume links	5	5	4	3	5	Batch process and audit
Agent duplicate/alias resolution	4	4	4	2	5	Automate candidate clusters
New referral capture	5	5	5	4	3	Escalate to IT; implement control
Old/no-evidence records	2	2	3	4	3	Defer or close with reason
Campaign on incomplete mapping	4	3	5	2	2	Stop until approval gate passes

5. Quick Wins

The fastest route to business impact is the first 20% by value, not necessarily 20% by record count. Validate whether the 136 priority agents account for a disproportionate share of active patients and revenue before claiming an 80% effect.

- Freeze scope and create one controlled register.
- Extract agent, registration, insurance, finance, and CRM/CDP sources with row counts and dates.
- Create canonical Agent IDs and duplicate clusters.
- Run a 100-row quality gate before scaling.
- Process the 136 priority queue and route uncertain records to exceptions.
- Make Agent ID/referral source mandatory at the first approved touchpoint.

6. AI Opportunities

AI can do	AI prepares; human checks	Human decision only
Name/code standardisation; duplicate detection; summaries; missing-field lists.	Candidate Agent-HN matches; confidence scores; priority ranking; conflict summaries.	Disputed attribution; campaign eligibility; reward/financial attribution; PDPA purpose/access/retention.

Safe operating rule

AI may suggest a link but must not independently decide patient attribution, change financial ownership, trigger benefits, or send communications. Patient identifiers should be minimised or pseudonymised before use in general-purpose AI. Human approval and an auditable source trail are mandatory.

7. Future-State Workflow

1 Capture	2 Validate	3 Enrich + score	4 Human review	5 Approve + load
Agent ID or Unassigned	Required fields + duplicate check	Join data; confidence score	Confirm or route exception	IT load; reconcile; monitor

Statuses: Unreviewed -> Candidate -> Confirmed -> Loaded -> Verified. Exceptions: Missing evidence / Duplicate / Conflict / Inactive / Closed-not-required.

8. 30-Day Action Plan

Timing	Action	Owner	Output
14-16 Sep	Freeze scope; request extracts; create canonical Agent ID.	CX manager + IT/data	Register and Agent master v1
17-20 Sep	Test 100 rows; process 136 priority agents; approve rules and PDPA boundaries.	Data team + manager + DPO/IT	Quality gate and exception queue
21 Sep-4 Oct	Batch process high-value links; load approved rows; reconcile.	Data team + IT	Loaded mapping and log
5-13 Oct	Launch mandatory capture, reminders, dashboard, weekly review, closure audit.	IT + Operations + sponsor	No new item >2 days; Day-30 report

9. KPI and Risk Controls

KPI	Current baseline	Target
Actionable backlog	Not yet validated; approx. 1,800 agents in scope	-50% in 30 days after denominator is defined
Average closure time	Not measured	-50% for standard cases
Error rate	Not measured	<5% audited sample; <2% after stabilisation
New unassigned records	Not measured	0 older than 2 working days
AI assistance	Not measured	60-80% of mechanical preparation

Early-warning triggers: blank Agent ID for two consecutive weeks; exception older than two working days; duplicate codes; personal LINE as primary evidence; unreconciled totals; or campaign requests before mapping audit passes.

10. Definition of Done

The backlog is closed only when every in-scope record has a status; every production mapping has a unique Agent ID, controlled patient key/HN, source evidence, reviewer, date, and confidence/reason code; duplicates/conflicts are resolved or assigned; approved rows are loaded and reconciled; the audit passes; and the new-referral process is live with no item older than two working days.

11. Final Recommendation

Approve the 30-day sprint immediately. The first deliverable should be a controlled, auditable mapping system showing what is confirmed, what is blocked, who owns each exception, and how new referrals are captured going forward.

The largest time saving will come from batch data preparation: standardisation, duplicate clustering, source joining, candidate matching, confidence scoring, exception summaries, and KPI reporting. Human judgement remains essential for attribution, privacy, financial impact, and final approval.

Sources and governance references

NIST AI Risk Management Framework 1.0 (2023): <https://www.nist.gov/itl/ai-risk-management-framework>

ICO Guidance on AI and data protection:

<https://ico.org.uk/for-organisations/uk-gdpr-guidance-and-resources/artificial-intelligence/guidance-on-ai-and-data-protection/>

Thailand Personal Data Protection Committee: <https://www.pdpc.or.th/>